

Eastmere General Hospital Patient Flow and Administration Handbook

Eastmere General Hospital is a fictional organisation, and this handbook was written as evaluation material.

Introduction

This handbook describes how patients move through Eastmere General Hospital, from the first contact with the hospital to the final settlement of the account. It is written for ward managers, administrative staff, clinical leads and finance teams who need one shared picture of the operating model. The hospital has around six hundred beds and serves a mixed urban and rural population across the Eastmere valley. The clinical content in this handbook is generic and describes administrative flow only. It is not clinical advice and it never replaces professional judgement or local clinical guidelines. No real patient data appears in any example, and every name, number and code used here is invented.

The handbook is a living document. The patient flow office maintains it, and every change is agreed at the monthly patient flow board before it is published on the staff intranet. Where this handbook and a departmental procedure disagree, staff should raise the difference with the patient flow office rather than choose one version on their own.

Context

The hospital opened in its current building in 1987 and has grown through several extensions, the most recent being a new theatre block and a dedicated stroke unit. Demand for emergency care has risen every year for the past decade, and winter pressure now lasts well into spring. At the same time, insurers have tightened the evidence they expect before they settle a claim, and late or incomplete coding has become a visible cost. A review by the internal audit team found that each department described patient flow in its own words, which made handovers slow and reporting inconsistent. The board asked for one vocabulary that clinical, administrative and finance staff could all use, and this handbook is the result.

The hospital also faces a workforce challenge. Many experienced ward clerks and coders are close to retirement, and much of what they know about the flow of work has never been written down. Newer staff often learn the process from whoever happens to be on shift, which leads to small local variations that add up to large delays across the whole hospital. The board sees a written, shared model of patient flow as a way to keep that knowledge, to train new staff faster and to give the finance team confidence that the clinical record will support every claim.

The guiding idea is simple. A patient should experience one hospital, not a sequence of departments. Every handover in this handbook therefore names the team that gives, the team that receives, and the record that travels between them.

Operating Model

Eastmere General Hospital runs patient admission, care pathways, discharge and billing. Each of these domains has a named executive lead who reports to the patient flow board every month. The domains are deliberately broad, so that a patient journey can be traced from the front door to the final invoice without gaps.

Eastmere General Hospital operates the patient administration system and the electronic patient record. The patient administration system holds identity, location and episode data for every patient. The electronic patient record holds clinical notes, observations, orders and results. A separate billing engine and a laboratory information system exchange messages with both platforms through the integration hub. Staff should treat the patient administration system as the single source of truth for where a patient

is and which episode is open.

Roles

The patient flow manager chairs the daily capacity meeting at eight in the morning and at four in the afternoon. The bed manager on duty holds the live view of every ward and makes the final call when two requests compete for the same bed. Triage nurses in the emergency department assess every new arrival and record the result in the electronic patient record. Pathway leads are senior clinicians who own the content of a care pathway and approve changes to it. Discharge coordinators work on each ward and chase the tasks that stand between a patient and a safe journey home. Clinical coders translate the clinical record into standard codes after each episode closes. Billing officers prepare and follow up claims to insurers and invoices to patients. The finance business partner reviews income against plan each month and reports variances to the board.

Key Performance Indicators

The patient flow board reviews a small set of indicators every month. The measures are chosen so that each domain has at least one indicator it can influence directly. Targets are reviewed each April and are set to be demanding but achievable.

Indicator	Owner	Target.
Arrival to triage time	Emergency department	Under fifteen minutes.
Decision to admit to bed time	Bed manager	Under four hours.
Discharges before noon	Discharge coordinators	Forty percent of discharges.
Coding completed within five days	Clinical coding team	Ninety-five percent of episodes.
First-pass claim acceptance	Billing office	Ninety percent of claims.

The arrival to triage time is measured from the moment the patient is booked in at reception. The decision to admit to bed time is the most visible indicator of flow, because a long wait in the emergency department usually means that beds upstairs are blocked. The share of discharges before noon matters because morning discharges free beds in time for the afternoon peak of admissions. Coding within five days protects income, since insurers reject claims that arrive late. First-pass claim acceptance shows how clean the claim file is when it leaves the hospital.

Patient Admission

Patient admission consists of elective admission, emergency admission, patient registration and bed management. Every admission, whatever its route, ends with a patient in a bed and an open episode in the patient administration system. The admissions office and the bed management team share the same ward view so that planned and unplanned demand are seen together.

Elective Admission

Elective admission consists of pre-admission assessment and admission booking. Elective patients are referred by a consultant after an outpatient appointment and join a waiting list for their procedure. The pre-admission assessment records baseline observations, which the ward team uses as a reference point after the procedure. The assessment also confirms that the patient understands the planned procedure and has arranged support at home. Admission booking produces an admission letter. The admission letter tells the patient the date, the ward, the time to arrive and the fasting instructions. Admission booking also raises a bed request, usually two working days before the admission date. When a booked patient cancels, the admissions office offers the slot to the next suitable patient on the list.

Emergency Admission

Emergency admission consists of triage and the admission decision. Patients arrive by ambulance, by referral from a general practitioner or on foot, and every arrival is triaged within minutes. Triage has five triage categories: immediate, very urgent, urgent, standard and non-urgent. The category sets the maximum time before the patient is seen by a doctor, and it is reviewed if the patient's condition changes while waiting. After assessment and initial tests, the senior doctor decides whether the patient needs to stay. The admission decision produces an admission request. The admission request names the receiving specialty, the level of care required and any isolation needs. The admission request triggers a bed request in the patient administration system.

Patient Registration

Patient registration consists of identity verification and insurance capture. Registration happens at the first contact for elective patients and as soon as it is safe for emergency patients. Identity verification assigns a hospital number, which stays with the patient for life. Staff check two independent identifiers before they confirm an existing record, to avoid creating duplicates. Insurance capture creates a coverage record. The coverage record holds the insurer name, the policy number and the authorisation code. For planned procedures the authorisation code must be present before the admission date, otherwise the billing office is alerted. Patient registration updates the patient administration system, so that every later step reads the same identity and coverage data.

Bed Management

Bed management consists of the bed request and bed allocation. Bed management maintains three bed pools: surgical beds, medical beds and critical care beds. The bed manager can lend a bed from one pool to another during periods of pressure, but the loan is recorded and reviewed at the next capacity meeting. Bed allocation produces a bed assignment. The bed assignment links the patient to a ward, a bay and a bed number in the patient administration system. Porters and ward staff are notified automatically when a bed assignment is made. A bed that has just been vacated is cleaned and inspected before it can be assigned again.

Care Pathways

Care pathways comprise the hip replacement pathway, the stroke pathway and order management. A care pathway sets out the expected sequence of care for a group of patients with a similar condition, so that teams know what should happen next and can see when a patient falls behind. Each pathway is owned by a pathway lead and is reviewed at least once a year. Pathways describe the usual journey; clinicians may always depart from a pathway when a patient needs something different, and they record the reason.

Hip Replacement Pathway

The hip replacement pathway consists of the pre-operative stage, the surgical stage, the post-operative stage and the rehabilitation stage. Most patients on this pathway are elective and arrive through the admission booking route. The pre-operative stage includes the anaesthetic review. The anaesthetic review requests a full blood count so that the team has a recent baseline before theatre. The surgical stage includes the surgical safety checklist and the hip arthroplasty. The surgical safety checklist consists of sign in, time out and sign out. The theatre team completes each part aloud, and the circulating nurse records it in the electronic patient record. The hip arthroplasty generates a theatre charge once the operation note is signed. The post-operative stage includes pain management, which the ward team reviews at every set of observations. The rehabilitation stage includes physiotherapy sessions, starting where possible on the day after surgery. The pathway expects most patients to go home within three days, although the actual length of stay depends on the individual.

Stroke Pathway

The stroke pathway consists of the hyperacute stage, the acute stroke unit stage and stroke rehabilitation. Time matters more on this pathway than on any other, and the hyperacute stage is designed to remove every avoidable delay. The hyperacute stage includes the stroke alert and brain imaging. The stroke alert is raised by the ambulance crew or by the triage nurse and calls the stroke team to the front door. Brain imaging requests a CT scan, which the radiology team prioritises over all routine work. The acute stroke unit stage includes the swallow screen. The swallow screen is completed before the patient is given food, drink or oral medication. Stroke rehabilitation includes speech and language therapy, and it often continues after discharge in the community. The pathway lead reviews every stroke admission at a weekly meeting to find where time was lost.

Order Management

Order management consists of order entry and result reporting. Order management serves every pathway, because almost every clinical step either places an order or waits for a result. Order entry creates laboratory orders and imaging orders. A full blood count is a kind of laboratory order. A CT scan is a kind of imaging order. Order entry takes place in the electronic patient record, which checks for duplicate requests before it sends the order on. Result reporting produces the result report. The result report is filed against the order and flagged to the requesting clinician, who must acknowledge it. Unacknowledged results are escalated to the clinical lead after twenty-four hours.

Discharge

Discharge consists of discharge planning and discharge completion. Discharge starts on the day of admission, not on the day the patient leaves, and the ward team discusses it at every board round. A delayed discharge affects the whole hospital, because it holds a bed that an emergency patient is waiting for.

Discharge planning includes the estimated discharge date and the needs assessment. The estimated discharge date is set within twenty-four hours of admission and is reviewed daily. The estimated discharge date informs bed allocation, since the bed manager plans tomorrow's capacity from today's expected discharges. The needs assessment looks at mobility, home circumstances and any support the patient will need after leaving hospital. When the assessment shows a need for care at home, the discharge coordinator contacts community services early.

Discharge completion consists of medicines reconciliation and the discharge summary. Medicines reconciliation compares the medicines the patient took before admission with the medicines they will take at home, and the ward pharmacist signs it off. The discharge summary contains three coded sections: primary diagnosis, procedures performed, discharge medications and follow-up plan. The summary is sent to the patient's general practitioner on the day of discharge. A copy is given to the patient in plain language. Patients who are ready to leave but waiting for transport or medicines move to the discharge lounge, which frees their ward bed earlier in the day.

Billing

Billing consists of clinical coding, charge capture, insurer claims and patient invoicing. Billing depends on every earlier domain, since a missing registration detail, an unsigned operation note or a late discharge summary each delays income. The billing office works closely with the clinical coding team and meets them weekly to clear backlogs.

Clinical Coding

Clinical coding consists of diagnosis coding and procedure coding. Coders start work once the episode is closed in the patient administration system. Diagnosis coding reads the discharge summary and the clinical notes to identify the main condition treated and any other conditions that affected care.

Procedure coding records each operation and significant intervention using the national classification. When the record is unclear, the coder sends a query to the responsible clinician rather than guessing. A sample of coded episodes is audited every quarter.

Charge Capture

Charge capture consists of charge event recording and tariff lookup. Charge event recording creates charge lines. A theatre charge is a kind of charge line. A bed-day charge is a type of charge line. Tariff lookup prices each charge line against the contracted rates for the patient's insurer or against the hospital's self-pay price list. Charges are captured as close as possible to the clinical event, because charges added weeks later are often challenged.

Insurer Claims

Insurer claims consist of claim preparation, claim submission and denial management. Claim preparation produces the claim file. Claim preparation reads the coverage record to confirm the policy number and the authorisation code before any claim is built. Claim submission uses two submission channels: the insurer portal, electronic data interchange and paper submission. Paper submission is kept only for the few insurers that cannot yet receive electronic claims. Denial management handles every claim that an insurer rejects or pays only in part. The billing officer checks the reason, corrects the claim where the hospital was at fault and appeals where the hospital believes the claim was valid.

Patient Invoicing

Patient invoicing consists of invoice generation and payment collection. Invoice generation produces the patient invoice. Invoice generation reads the charge lines and subtracts any amount the insurer has agreed to pay. A self-pay invoice is a kind of patient invoice. Payment collection offers several ways to pay, including instalments for larger balances. Patients who find it hard to pay are referred to the patient support team, and no patient is ever refused care because of an unpaid invoice.

Using This Handbook

Teams should use the terms in this handbook in handovers, reports and system change requests. New terms are proposed to the patient flow office and agreed at the patient flow board before they are used in shared reports. The handbook will be reviewed in full every year, and sooner if a major system or contract changes. Comments and suggestions are welcome from any member of staff, whatever their grade or department. The most useful feedback describes a real handover that went wrong, what the team expected to happen and what actually happened, so that the patient flow office can see where the written model and daily practice have drifted apart.